

Provider-Specific Amendment/Rider

Blue Cross Blue Shield - ABC Integrated Health Network Benefit Plan Amendment ID: AMND-2025-0352-ABC Issue Date: November 15, 2024

Amendment Scope and Effective Date

This Amendment is issued to modify certain provisions of the health benefit plan contract between Blue Cross Blue Shield (hereinafter "BCBS") and the ABC Integrated Health Network (the "Provider Group"). It shall be binding upon both parties and shall be read in conjunction with the original policy document, specifically amends document id `w14\_bluecross\_blueshield\_detailed\_policy\_038`, particularly the amended section: **Definitions and General Provisions**, as well as related coverage schedules and benefit descriptions that flow from those definitions. The effective date of this Amendment is January 1, 2025, at 12:00 a.m. local time at the subscriber's address of record. All terms not expressly altered herein remain in full force and effect, subject to the supersession list in Section 3 (Superseded Provisions) of this document. This Amendment addresses updates to cost-sharing structures, utilization management protocols, and certain benefit-specific parameters that the parties have negotiated specifically for the ABC Integrated Health Network's participating practice locations (Tax ID 45-6789012) and all eligible dependents covered under the same master group policy. In consideration of the Provider Group's agreement to participate in advanced value-based care models, BCBS is adjusting the standard benefit design for the associated subscriber population. The modifications below supersede any conflicting information in the Certificate of Coverage, Summary of Benefits and Coverage (SBC), and any prior riders, addenda, or correspondence. The waiting period for new enrollees shall be determined solely by the group's probationary period as defined in the master contract; **pre-existing waiting: None** applies to this plan, meaning no pre-existing condition limitation or exclusion shall be imposed irrespective of prior creditable coverage or break in coverage, consistent with the Affordable Care Act and the plan's grandfathered/non-grandfathered status as applicable^[^1]. The following section details the specific alterations to member cost-sharing and utilization management rules, incorporating a mix of descriptive text and tabular summaries for ease of reference but presented without strict formal ordinal numbering to reflect the negotiated nature of this rider.

[^1]: The elimination of pre-existing condition waiting periods is subject to the definition of "pre-existing condition" as interpreted under HIPAA and ACA. This provision does not override any enrollment period restrictions that apply to late enrollees; late enrollees may still be subject to the group's standard waiting period for entry into the plan, which is not a pre-existing condition limitation.

Modified Coverage Terms

1. In-Network Cost-Sharing Adjustments - The following table replaces the in-network deductible, coinsurance, and out-of-pocket maximum language found in the Policy's Benefit Summary for the ABC network providers. All amounts are on a calendar-year basis per individual unless family aggregation applies. The family deductible is twice the individual amount for embedded deductible structures, and family out-of-pocket maximum follows a true family limit as defined in the plan's mathematical aggregation formula (not merely an aggregate of individual limits). Services rendered by Providers within the ABC Integrated Health Network (Tier 1) shall be subject to these amounts; out-of-network benefits are addressed separately. The new amounts are effective for dates of service on or after the effective date, and run-out claims for prior periods shall adjudicate under the prior schedule except as noted in Section 3.

Cost-Share Component	New Value
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in_network_deductible: \$1,200	Individual Medical Deductible (excludes certain preventive services)
out_of_pocket_max: \$5,800	Individual Out-of-Pocket Maximum (includes deductible, copays, coinsurance for covered services; does not include premiums, balance-billing charges, or non-covered services)
Emergency Room Copayment	er_copay: \$200 (waived if directly admitted as inpatient from the ER; copay applies per visit and is in addition to any facility deductible/coinsurance if applicable)
Mental Health Outpatient Visits (Individual)	mental_health_visits: 30/year, \$35 copay per visit (combined limit for psychotherapy, psychological testing, and medication management; visits beyond 30 are covered at 50% coinsurance after deductible with medical necessity review, unless otherwise mandated by state parity law)
MRI Prior Authorization	mri_prior_auth: Not required for in-network; however, advanced imaging like CT scans and PET scans still require preauthorization through the BCBS radiology management program. Out-of-network MRI remains subject to prior authorization as per standard plan rules.

The in-network deductible accumulates separately from the out-of-network deductible. Prescription drug copayments and the ER copay do not count toward the in-network deductible but do accumulate toward the out_of_pocket_max: \$5,800. The plan maintains an integrated medical and pharmacy out-of-pocket maximum, meaning allowed expenses from both medical and prescription drug services under the pharmacy benefit manager (if carved in) count toward the same \$5,800 limit. For members whose coverage includes a Health Reimbursement Arrangement (HRA) funded by the employer, the employer's contributions may be used to satisfy the deductible, but the member must still meet the full deductible through payments or account draws before the plan begins paying non-preventive benefits at the negotiated coinsurance rates. The standard in-network coinsurance remains 20% after deductible unless a service is listed as copay-only.

2. Out-of-Network Coverage - For services received outside the ABC Integrated Health Network, out_of_network_coverage: 70% after \$2,500 deductible. This deductible is separate and does not cross-accumulate with the in-network deductible. The out-of-network out-of-pocket maximum is \$12,000 (individual), and any balance billing above the allowed amount is the member's responsibility and does not accrue toward that maximum. The allowed amount for out-of-network claims is based on the lesser of the provider's actual charge or the BCBS reference-based pricing schedule, which is derived from 140% of the Medicare fee schedule for similar services in that geographic area, subject to a minimum reasonable and customary data review. Claims for out-of-network emergency services are processed at the in-network level (i.e., \$200 copay and in-network deductible/coinsurance) only when the member could not reasonably access an ABC network facility; otherwise, the out-of-network cost-sharing applies. This is consistent with the No Surprises Act, but members should be aware that post-stabilization care after an ER visit may be subject to balance billing if received out-of-network without authorization. We reiterate that the pre_existing_waiting: None applies uniformly regardless of network tier; there is no exclusion for pre-existing conditions even when seeking out-of-network care, as long as the plan otherwise covers the service.

3. Prescription Drug Tiers - The plan uses a formulary managed by BCBS's PBM, with a tiered copayment structure. The prescription drug benefit includes prescription_tiers: 4 tiers, described as follows. Tier 1: Preferred generic drugs (copay \$10, retail 30-day supply; mail order 90-day supply at \$20). Tier 2: Preferred brand-name drugs (\$35 copay retail, \$70 mail order). Tier 3: Non-preferred brand-name and select specialty drugs (\$70 copay retail, \$140 mail order, or 30% coinsurance for specialty drugs administered in a provider's office). Tier 4: High-cost specialty and biotech drugs (coinsurance 30% up to a maximum copay of \$250 per fill, with coordination of benefits through the specialty pharmacy network). The plan also includes a preventive drug list where certain Tier 1 and Tier 2 drugs are covered at no member cost-sharing if dispensed by an in-network pharmacy and prescribed for preventive indications as defined by USPSTF A/B recommendations. The prescribing provider must be part of the ABC network for the member to receive the network pharmacy benefit; otherwise, out-of-network pharmacy charges apply (50% coinsurance after a separate \$500 out-of-network pharmacy deductible).

Mental Health Parity Note: The covered mental health outpatient visits (30/year, \$35 copay) include telehealth sessions delivered by an ABC network mental health provider; these are not subject to the deductible and count toward the visit limit. Any visit, regardless of modality, that is rendered by a non-network mental health provider will be reimbursed under out-of-network mental health benefits (50% coinsurance after a \$1,000 out-of-network deductible for behavioral health, separate from medical out-of-network). This out-of-network mental health deductible is embedded in the \$2,500 medical out-of-network deductible, meaning the member does not need to satisfy an additional deductible for mental health beyond the general non-network amount.

4. Other Coverage Modifications - Certain sections regarding preventive care, diabetic supplies, and rehabilitation therapy have been revised to align with the ABC network's value-based care contracts. Physical therapy, occupational therapy, and speech therapy are combined into a single rehabilitation benefit of 45 visits per calendar year, with a \$25 copay per visit at in-network providers, no deductible. The previous 60-visit separate limits are superseded. For bariatric surgery, the plan now uses a Centers of Excellence requirement: services must be performed at an ABC-designated facility, and prior authorization remains required. For organ transplants, the benefit is covered at 100% after deductible at a Blue Distinction Center for Transplants, provided pre-certification is obtained. Additionally, coverage for applied behavior analysis (ABA) therapy for autism spectrum disorder is subject to the mental_health_visits: 30/year, \$35 copay but with a separate authorization process specific to developmental disabilities, not counting against the general rehabilitation visit limit.

All these amendments are subject to the general exclusions and limitations of the policy, including experimental/investigational definitions, medical necessity criteria, and the coordination of benefits provisions. The following long paragraph serves as a clarification on how these new terms intersect: The accumulation of out-of-pocket expenses across plan components shall be administered via a real-time accumulator system that recognizes both the medical deductible (in_network_deductible: \$1,200) and the pharmacy deductible (embedded in the \$1,200 when pharmacy claims are processed through the medical benefit as under the carved-in PBM arrangement; if pharmacy is carved out, a separate pharmacy deductible of \$250 applies for tiers 3-4 only). For families, the overall out-of-pocket maximum may be reached by any combination of individuals; however, no single individual will be required to pay more than \$5,800 in cost-sharing within a given year. The emergency room copayment (er_copay: \$200) is subject to the out_of_pocket_max: \$5,800, meaning once the member hits the maximum, the ER copay is waived. This also applies to the mental health visit copayments, but those copayments do not apply toward the deductible as noted. It is the member's responsibility to ensure that providers are in the ABC network at the time of service; BCBS provides an online directory updated weekly, but the ultimate determination of network status is based on the provider's contract with BCBS at the date of service. The plan will reprocess claims if a provider was incorrectly listed as in-network,

holding the member harmless from any extra cost beyond in-network cost-sharing.

Superseded Provisions

The following specific items from the original policy document, w14_bluecross_blueshield_detailed_policy_038, and all prior riders are hereby replaced in their entirety by the terms of this Amendment. Where a reference is made to a section, it refers to the policy's internal numbering as it existed prior to this Amendment. The amended section "Definitions and General Provisions" is overhauled to incorporate the new benefit-maximum definitions, such as "Rehabilitation Visit Limit", "Mental Health Outpatient Visit Limit", and "Out-of-Network Deductible" as defined herein. Additionally, the previous Benefit Summary's table for medical cost-sharing that listed a \$1,500 individual in-network deductible, \$6,500 out-of-pocket maximum, \$150 ER copay, and 20 outpatient mental health visits at \$40 copay is entirely superseded. The prior prescription drug schedule that had only 3 tiers (generic, preferred brand, non-preferred) is replaced with the 4-tier structure. The old language that required prior authorization for all MRI studies, irrespective of network, is deleted and replaced with mri_prior_auth: Not required for in-network. The previous existence of a 12-month pre-existing condition exclusion rider for late enrollees is removed; the policy now states pre_existing_waiting: None. Coverage for out-of-network services was previously written as 60% coinsurance after a \$3,000 deductible and is now changed to out_of_network_coverage: 70% after \$2,500 deductible, with a corresponding adjustment to the allowed amount methodology. Any reference to "mental health outpatient visits" in the Certificate of Coverage is updated from "20 visits per year" to "mental_health_visits: 30/year, \$35 copay". The amendment also supersedes Section 8.2 (Emergency Services) to insert the er_copay: \$200 and clarifies the waiver conditions. All other plan provisions not in conflict remain unchanged, including eligibility rules, termination for cause, subrogation, and grievance procedures. This Amendment constitutes the entire agreement between the parties regarding these specific modifications and may only be further amended by a written instrument signed by both BCBS and the Provider Group.

This document is a legally binding amendment to the health benefit plan. If any provision is found to be inconsistent with state or federal law, the law shall prevail and the remaining provisions shall be enforced.

For Blue Cross Blue Shield: /s/ Authorized Representative Date: _____

For ABC Integrated Health Network: /s/ Authorized Representative Date: _____